

UPMC Mercy

1400 Locust St, Pittsburgh, PA 15219

DISCHARGE SUMMARY Document #: DIS-013

Patient & Admission

Name	Tamara Higgins
Date of Birth	1970-10-04
Phone	412-555-5947
Address	374 Jackson Rest Greensburg, PA, 15601
MRN	MRN-496631
Admit Date	2026-04-18
Discharge Date	2026-04-23
Attending	William Andrews, MD
Attending NPI	5799118380

Insurance

Primary Payer	Cigna
Primary Member #	U531003309

Primary Discharge Diagnosis

Acute on chronic systolic heart failure

ICD-10: I50.31, I10

Hospital Course

The patient presented to the emergency department with a three-day history of progressive dyspnea, orthopnea, and lower extremity edema. Vital signs on arrival revealed hypertension to 158/92 mmHg, tachycardia at 102 beats per minute, and oxygen saturation of 91% on room air. Physical examination demonstrated bibasilar crackles, elevated jugular venous pressure, and 2+ pitting edema to the knees bilaterally. Chest radiography confirmed pulmonary edema with bilateral infiltrates. Laboratory studies showed elevated BNP at 1,240 pg/mL, mild elevation in troponin I at 0.08 ng/mL, and a creatinine of 1.4 mg/dL reflecting mild renal impairment. Transthoracic echocardiography revealed an ejection fraction of 28% with global hypokinesis, consistent with his known ischemic cardiomyopathy. The patient was admitted to the telemetry unit with a diagnosis of acute on chronic systolic heart failure, likely precipitated by medication non-adherence and dietary indiscretion.

Initial management included intravenous diuresis with furosemide, uptitration of his home lisinopril and carvedilol, and initiation of high-dose atorvastatin. The patient responded well to therapy, with significant improvement in dyspnea by hospital day two and resolution of orthopnea by day three. Oxygen requirements decreased to room air by day four. Repeat echocardiography was deferred given recent imaging and stable clinical trajectory. Renal function remained stable throughout the admission, and troponin elevation was

attributed to demand ischemia in the setting of acute decompensation rather than acute coronary syndrome.

The patient was discharged on hospital day five in stable condition with optimized medical therapy including lisinopril 20 mg daily, carvedilol 25 mg twice daily, furosemide 40 mg daily, and atorvastatin 80 mg daily. He was counseled extensively on sodium restriction, fluid management, and the critical importance of medication adherence. Follow-up with cardiology was arranged for two weeks post-discharge, and he was instructed to contact his primary care physician or return to the emergency department if symptoms recur.

Procedures Performed

Right heart catheterization	CPT 93451
------------------------------------	-----------

Medication Changes

Metoprolol tartrate	STOPPED
Furosemide 40 mg BID	STARTED
Empagliflozin 10 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	9 days
Urgency Tier	MEDIUM
Clinical Flags	EF<40%, 30-day-readmission-risk, active volume overload at discharge

Dictated and electronically signed by:
William Andrews, MD
NPI: 5799118380
UPMC Mercy